



Transthoracic Echocardiography Report

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MR Number	: 00261280	Study Date	: 11 August 2025
Date of Birth	: 20/03/1939	Diagnosis	: AF NVR
Age	: 86 y.o	Referring Physician	: dr. Gusti Ayu Temi, SpJP(K)
		Medical Device	: Philips CVx 3D

Measurements & Calculations								
BSA		m ²	IVSd	10.2	mm	MV E/A ratio		
Ao diameter	28	mm	LVIDd	30.1	mm	MV E Vel		cm/s
LA diameter	44	mm	LVPWd	14.6	mm	MV A Vel		cm/s
LVOT diameter		mm	IVSs	10.6	mm	MV e' septal		cm/s
LVOT VTI		cm	LVIDs	17.9	mm	MV e' lateral		cm/s
EF (teich)	72.9	%	LVPWs	14.2	mm	E/e' average		
EF (Simpson's)	65.6	%	LVMI	73.0	gr/m ²	PV AccT		ms
TAPSE	1.80	cm	RWT	1.0		LAVi	74.2	gr/m2

Findings	
ECG	Atrial fibrillation
Study Quality	The quality of the study is good.
Left Ventricle	There is concentric remodelling. Global LV systolic function is normal, LVEF 65.6% (Simpson's).
Right Ventricle	Wall thickness of the RV is normal. RV systolic function is normal, TAPSE 1.80 cm.
Left Atrium	LA size is dilated, LAVI 74.2 gr/m2.
Right Atrium	RA size is normal.
Interatrial Septum	Intact interatrial septum
Interventricular Septum	Intact interventricular septum
Aortic Valve	The aortic valve is trileaflet and opens well. There is no calcification. There is no AR.
Mitral Valve	There is flail of AML with severe MR, VC width 0.7 cm. Regurgitant jet is eccentric. There is no MS.
Tricuspid Valve	Tricuspid valve opens well. There is mild TR, VC width 0. cm. TR Vmax 3.05 m/s, TVG 37 mmHg.
Pulmonic Valve	Pulmonic valve appears normal.
Mass / Thrombus	There is no abnormal mass or thrombus.
Pericardium	Pericardium is normal.
2D Wall Motion	Hypokinetic in basal-mid anteroseptal and apicoseptal LV.

CONCLUSION

Moderate functional MR
Moderate TR with intermediate probability of PH
Concentric LVH with reduced LV systolic function, LVEF 47.4% (Biplane)
Hypokinetic in basal-mid anteroseptal and apicoseptal LV
Good RV contractility

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